Quality Documentation Toolkit | Templates and Review Checklist

Reusable fictional forms • DCR • Exception log • Training / change / review records

TRAINING EXAMPLE ONLY — Fictional organization, synthetic records, and invented personnel. Not a validated procedure, production instruction, executed approval, or assertion of regulatory compliance.

A. Document change request (DCR) — blank template

Field	Entry / prompt
DCR number	[Assign unique controlled ID]
Submitted date / requester	[Date / name]
Document / version affected	[Document ID and effective revision]
Reason for change	[Observed issue, requirement, proposed improvement]
Change summary	[Precise sections and intended edits]
Risk and impact	[GMP, QC, material, system, reporting, training and user impact]
Required reviewers	[Process owner, QA, QC, Procurement, IT as applicable]
Disposition / signature reference	[Pending/approved/rejected; link authorized record]
Implementation/verification	[Effective date, publication, training, post-change check]

B. Source-to-system exception log — blank template

Field	Entry / guidance
Exception ID / logged at	[Identifier, time, reviewer]
Source document / field	[Evidence location and record ID]
Observed / expected	[Exact values and controlled expected source]
Severity / priority	[Based on authorized risk rubric]
Current owner / next action	[Role, due date, required clarification]
Status / ageing	[Open, pending review, verification, closed]
Evidence / approval	[Link, not a made-up signature]
Closure rationale	[Confirmed correction or documented no-change rationale]

C. Training-impact assessment — blank template

Question	Reviewer response
Who uses the revised instruction?	[Roles, sites, groups]
What changes in behavior?	[New/modified step]
How is competence demonstrated?	[Read-and-acknowledge, practical demonstration, test, as approved]
Who assigns and tracks training?	[Training owner/system]
When must training finish?	[Before affected work where required]
What record is retained?	[Training evidence identifier]

D. Release-ready document QA checklist

1. Metadata: document ID, version, owner, status and referenced forms agree.
2. Procedure: tasks have actor, trigger, input, action, output and decision or escalation.
3. Regulatory wording: references are precise; no overbroad compliance promises.

4. Evidence: reviewer comments, approvals and training plan linked and available.
5. Traceability: source data, change log, effective version and archived revision remain reconstructible.
6. Accessibility: descriptive headings, readable tables, no ambiguous "click here," consistent abbreviations.
7. Publication: effective date assigned only by authorized workflow; no unapproved draft at point of use.

E. Hypothetical register examples

ID	Record	Status	Responsible role	Notes
SOP-QA-014	Document lifecycle SOP	DRAFT	Quality Systems	No effective date
WI-QA-021	Vendor/material data QC	DRAFT	QA Operations	Example only
EX-2026-018	Vendor ID / specification discrepancy	OPEN	QA Operations	Material remains hold
DCR-2026-007	Revision to verification workflow	OPEN	Process Owner	Pending review
CAPA-2026-003	Vendor mapping preventive controls	PROPOSED	Quality Systems	Not executed

F. Evidence-to-claim discipline

When preparing a public technical-writing portfolio, never turn a proposed control into an executed fact. Label statuses accurately and separate example effectiveness acceptance criteria from measured results. Do not publish customer/vendor identities, employee signatures, batch numbers, controlled company SOPs, patient information, or screen captures from actual regulated systems.

REGULATORY / TECHNICAL REVIEW ADDENDUM — November 2026

Independent technical writing portfolio enhancement. Additional validation gates and examples are illustrative, not authorized production instructions. Regulated applicability varies by product, process, record and organization.

Document change request: complete sample schema

Required fields: request ID; document ID and revision; initiating role; reason and problem statement; source evidence; impacted site/process; affected supporting records; proposed text; risk assessment owner; QC/Manufacturing/Procurement reviewer list; electronic record assessment; training impact; approval decision; proposed effective date; supersession/retention disposition.

Example DCR-2026-007: current SOP-QA-014 v1.0, proposed v1.1; reason: ambiguous vendor-ID matching step; impacted record WI-QA-021; QA and Procurement review pending; training impact UNDER ASSESSMENT; approval state NOT APPROVED.

Evidence-based exception and CAPA templates

Exception fields: record ID, discovery timestamp, source system, field name, observed value, expected value/reference, preliminary risk, containment/lot status, assignee, response dates, verification evidence, authorized decision and closure date. Never insert personal or patient data into a public example.

CAPA fields: investigation statement, root-cause rationale, evidence reviewed, immediate containment, correction, corrective/preventive action owners, change-control reference, effectiveness method, observation period, acceptance rule and QA review outcome.

Publication, integrity and accessibility checklist

Check title, owner, revision, status, effective date, approved forms, referenced documents, correct role authority, legible font, running header/footer and accessible tables. Use text labels such as HOLD, APPROVED and DRAFT rather than color alone.

Ensure printed black-and-white copies remain understandable; a blue heading is a presentation convention, not a CGMP mandate. Blue or black permanent ink preferences for hand-completed records must come from an actual controlled procedure, not inferred from the color of portfolio PDFs.

Document-specific reference basis and status

FDA 21 CFR 211.22 — quality unit authority (full URL in portfolio review report)

FDA 21 CFR 211.84 — component sampling/testing/release (full URL in portfolio review report)

FDA 21 CFR Part 11 — electronic records and e-signatures (full URL in portfolio review report)

FDA Part 11 scope and application guidance (full URL in portfolio review report)

FDA Data Integrity and Compliance With Drug CGMP (2018) (full URL in portfolio review report)

ICH Q9(R1) Quality Risk Management (2023) (full URL in portfolio review report)

ICH Q10 Pharmaceutical Quality System (full URL in portfolio review report)

ICH Q8/Q9/Q10 Q&A R5 (May 2026) (full URL in portfolio review report)

Status: technical/editorial review against publicly available references completed at a portfolio-example level; no licensed-standard full-text conformance audit, live environment testing, site QA approval or legal certification performed.

Appendix: November 2026 standards and documentation applicability review

PUBLIC SYNTHETIC PORTFOLIO SAMPLE — This appendix records source-checking and editorial scope boundaries. It does not make the fictional organization regulated, certify a system, or replace actual company policies and competent quality/legal review.

Pharmaceutical regulatory scope and data-integrity editorial verification

- 21 CFR 211.22: fictional QA unit responsibility is distinct from QC testing and procurement approval; incoming component disposition must not be inferred from data-field reconciliation alone.
- 21 CFR 211.84: describe quarantine, identity testing and QC release expectations only to the extent that the hypothetical material falls under the rule; signed review of a supplier certificate is not automatic approval.
- 21 CFR 211.180/211.192 and FDA data-integrity guidance: reconcile original records, trace corrections, record investigations, preserve legibility and demonstrate review of unexplained discrepancies.
- 21 CFR Part 11: evaluate record and electronic-signature predicate-rule scope; do not claim that every spreadsheet or SOP falls under Part 11.
- ICH Q9(R1)/Q10 and FDA May 2026 Q8/Q9/Q10 Q&A: risk-based investigation, change control, oversight, effectiveness checks and management review are guidance/model concepts, not blanket product approvals.
- 2026 FDA QMSR/21 CFR 820 and February 2026 Computer Software Assurance: relevant to in-scope medical-device workflows; not universally required for this hypothetical drug-component process.
- Document design check: explicit document ID/version/status, role-based responsibilities, objective acceptance criteria, accurate dates, no invented e-signatures or final QA approvals, and clear synthetic label.

Reference links and recheck notes

https://www.ecfr.gov/current/title-21/chapter-I/subchapter-C/part-211

https://www.ecfr.gov/current/title-21/chapter-I/subchapter-A/part-11

https://www.fda.gov/regulatory-information/search-fda-guidance-documents/data-integrity-and-compliance-drug-cgmp-questions-and-answers-guidance-industry

https://www.fda.gov/medical-devices/postmarket-requirements-devices/quality-management-system-regulation-qmsr

Verified against published authoritative source pages as of November 2, 2026. No proprietary standards were audited clause-by-clause. Actual application must be assessed based on product, system use, region, and record type. Proposed procedures have not been executed in a production environment.

Independent second-pass document review | November 2, 2026

REVIEWED PORTFOLIO DEMONSTRATION — this appendix records a document-specific editorial and source-scope check. The main procedure and forms are fictional and not approved for production, executed testing or regulated use. Original source versions are retained as editable files in this portfolio.

A. Document-specific findings and checks

Review objective	2026 reviewer correction / expectation	Verification evidence
Form controls	Include form identifier, controlled document revision, preparer/date, reviewer/approver, affected IDs, status and retention location.	A blank template is not a controlled approved record until reviewed under a real QMS.
Change/exception linkage	Demonstrate relationship between deviation, CAPA, material change request and training impact.	Cross-referenced fictional IDs are consistent with registers; changes cannot overwrite historical evidence.
Review checklist	Add mandatory field completion, role authority, traceability, version and print readability check.	A reviewer can identify unresolved cases without relying on color coding.

B. Applicable regulatory / guidance boundaries

Review objective	2026 reviewer correction / expectation	Verification evidence
Drug CGMP / Quality unit	21 CFR 211.22, 211.80, 211.84, 211.100, 211.160 as relevant	Mandatory only when the fictional activities actually fall within covered finished-drug operations; paperwork precheck is not QC material disposition.
Records integrity / electronic signature	21 CFR 211.180, 211.188, 211.194; Part 11 conditional	Trace data to author, time, source and approved version; do not assume that every spreadsheet is a Part 11 record.
Risk and change	ICH Q9(R1), ICH Q10; FDA 2018 final data integrity guidance	Nonbinding guidance/system model as adopted or implemented; distinguish a risk-based recommendation from regulation.

C. Evidence and sign-off boundaries

Status: Editorial second-pass reviewed; technical procedure not executed; company-specific SOP approval not obtained.

All tests, screenshots, users, suppliers, dates, IDs, incident observations, performance metrics and signatures in the demonstration are synthetic.

Next practitioner action: confirm source applicability for the actual jurisdiction and product; conduct SME/user testing and accessibility review; update controlled revision and approval fields.

No color, watermark, revision number, PDF export or reference link by itself demonstrates regulatory compliance.

D. Authoritative primary-source references

FDA drug CGMP 21 CFR Part 211: https://www.ecfr.gov/current/title-21/chapter-I/subchapter-C/part-211

FDA 21 CFR Part 11 scope: https://www.fda.gov/regulatory-information/search-fda-guidance-documents/part-11-electronic-records-electronic-signatures-scope-and-application

FDA drug CGMP final data integrity: https://www.fda.gov/regulatory-information/search-fda-guidance-documents/data-integrity-and-compliance-drug-cgmp-questions-and-answers

ICH Q9(R1)/Q10: https://admin.ich.org/page/quality-guidelines

FDA device QMSR Feb 2026: https://www.fda.gov/medical-devices/postmarket-requirements-devices/quality-management-system-regulation-qmsr